

Pain Management

History

- When did pain start? What was patient doing at onset of pain? What does pain feel like? (Pain Scale)
- Location of pain?
- Was there trauma?
- Is there altered mental status or reason to suspect head injury?
- Is the patient intoxicated?
- Does patient have nausea / vomiting?
- What medications does patient take?
- Does the patient have any known medication allergies?

Physical

- Mental Status – GCS < 15?
- Abnormal Vital Signs
 - Tachycardia and/or hypotension?
 - Hypertension – severe pain, cardiac?
 - Hypoxia – SpO₂ < 94%
 - Other signs of respiratory depression? Bradypnea / hypoventilation?
- Skin – Pale? Hot? Diaphoretic?
- 12 – Lead EKG if indicated

Some Considerations Before Administering Pain Medication

- Encourage oral or IV hydration after giving oral or IV pain medications that effect liver or kidneys
- Do not allow someone to operate machinery after giving sedating medications

EMT/AEMT

- Follow General Medical Care Guideline
- Non-narcotic pain management
 - Position of comfort – as appropriate
 - Splint injured extremities per appropriate guideline
 - Calm, quiet environment
 - Cold packs for acute injuries heat packs may be helpful for chronic muscle spasm.
- Obtain and record pain scale and vital signs every 5 minutes

EMT/AEMT - Expanded Scope

- Acetaminophen (PO), Ibuprofen (PO)/Ketorolac (IM), Lidocaine Patch (See paramedic section for doses)

Paramedic

- **Acetaminophen** 975mg PO or 650mg PR
 - May repeat every 6 hours as needed
 - Max daily dose 4g/day
 - Limit use to 1 dose in known liver disease or chronic alcoholic patient population. Consult medical direction for additional doses.
- **Ibuprofen** 400mg PO or **Ketorolac** 15mg IM or IV (Do not use together or within 6 hours of each other)
 - May repeat every 6 hours
 - Max daily dose 120mg/day; Max duration is 5 days
 - Avoid use in severely dehydrated or concern for rhabdomyolysis, severe burn patients, known renal disease
 - If giving Ketorolac, patient must be seen in a clinic or ED prior to returning to work
- **Lidocaine patch** 3.5% or 4% patch
 - Apply 1 patch to painful area
 - Patch may remain in place for 12 hours in any 24 hour period
 - No more than 1 patch should be used in a 24 hour period
- **Cyclobenzaprine** 10 mg PO for muscle spasm
 - In liver disease, use 5mg PO as the initial dose
 - May repeat dose every 8 hours as needed
 - Do not use for more than 3 days
 - This medication may be sedating. **Do not allow operation of heavy machinery, driving, or firefighting while using this medication**
 - May cause **Serotonin Syndrome: Avoid use** or contact medical director for patients taking one or more serotonergic agents:
 - SSRI (-lopram, -xetine, -xamine, -sertraline, -vilazodone)
 - SNRI (-afaxine, -xetine, -nacipran)
 - TCA (-triptyline, -pramine, -amoxapine)
 - Buspirone
 - Meperidine
 - Tramadol
 - MAOI (-phenelzine, -tranylcypromine, -selegiline, -isocarboxazid)
 - Bupropion
 - Opioids
- **Droperidol** 0.625-2.5mg IV/IO for pain uncontrolled by other means